Antibiotic Prescribing Policy

Reference	PHA-ANT-01 v7
Owner	Antimicrobial Stewardship Group
Approved by	Quality and Patient Safety Committee
Effective date	16/02/2026
Next review	16/02/2029
Applies to	All clinical areas of the Centre Hospitalier Saint-Aubin

1. Principles

Prescribe according to the indication tables below, document the indication and the planned duration on the drug chart, and review every antibiotic at 48 to 72 hours (start smart, then focus). Intravenous treatment is reviewed daily for a switch to oral.

2. Penicillin allergy

Record the nature of the reaction. A **severe or immediate reaction** (anaphylaxis, angioedema, urticaria with hypotension or bronchospasm) is a contraindication to every penicillin, including amoxicillin, co-amoxiclav and piperacillin-tazobactam. A non-severe, delayed rash allows a cephalosporin after discussion with the pharmacist.

The allergy status must be copied from the admission note to the drug chart on admission. A discrepancy between the two is a reportable medication incident.

3. Community-acquired pneumonia (CAP)

Situation	First-line regimen	Route
Moderate CAP, no allergy	Amoxicillin 1 g three times daily	oral
Moderate CAP, severe penicillin allergy	Doxycycline 200 mg on day 1, then 100 mg once daily	oral
Severe CAP	Ceftriaxone 2 g once daily (plus clarithromycin 500 mg twice daily)	intravenous

Total duration of treatment is 5 days. **Intravenous therapy is limited to 3 days**; from day 4 the patient must be switched to the oral regimen unless the infectious diseases team documents a reason to continue.

4. Urinary tract infection

Situation	First-line regimen	Duration
Lower, female	Nitrofurantoin 100 mg modified-release twice daily	3 days
Lower, male	Trimethoprim 200 mg twice daily	7 days
Pyelonephritis	Co-amoxiclav 1.2 g three times daily IV, then oral	7 days

5. Surgical antibiotic prophylaxis

Item	Requirement
Agent (clean-contaminated abdominal surgery)	Cefazolin intravenous
Dose	2 g; 3 g if body weight is 120 kg or more
Timing	within 60 minutes before skin incision
Re-dosing	repeat the dose if the operation lasts more than 4 hours or blood loss exceeds 1,500 mL
Duration	single dose; no post-operative continuation for clean-contaminated cases

The time of administration and the time of incision are both recorded on the anaesthetic record. Each month the audit team checks compliance with dose and timing and reports it to the surgical directorate.

6. Skin and soft-tissue infection

Situation	First-line regimen	Duration
Cellulitis	Flucloxacillin 1 g four times daily	5 days
Cellulitis, severe penicillin allergy	Clarithromycin 500 mg twice daily	5 days

7. Governance, audit and training

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8. Revision history

Version	Date	Author	Summary of change
1.0	16/02/2021	Antimicrobial Stewardship Group	First issue
2.0	16/02/2023	Antimicrobial Stewardship Group	Triennial review; format harmonised
3.0	16/06/2025	Antimicrobial Stewardship Group	Tables updated after clinical audit
current	16/02/2026	Antimicrobial Stewardship Group	Review and re-issue

Annex A. Implementation notes

Equality and diversity: this document has been assessed for its effect on patients and staff with protected characteristics. Reasonable adjustments, interpreters and accessible formats are available through the patient experience team.

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